

corpus cavernosum smooth muscle necrosis, followed by fibrosis manifesting as erectile dysfunction and penile shortening. Early insertion of a malleable penile prosthesis is easier to perform and also allows the option of an interval exchange to an inflatable penile prosthesis after a few months. The aim of this study was to assess whether upsizing of cylinders is still possible at the time of exchanging to an inflatable prosthesis.

Materials and Methods: Over a 24 month period 10 patients with refractory ischaemic priapism underwent an early (within 2 weeks) insertion of a malleable (Coloplast Genesis®) penile prosthesis. The mean age was 42.7 years (range 35-69) and the median duration of priapism was 7 days (range 5-10). The aetiology was sickle cell disease (2 patients), idiopathic (4 patients) and antipsychotic agents (4 patients). Following a median period of 5.7 months (range 3.8-11.5) all of these patients underwent exchange of their penile prosthesis to a 3 piece inflatable (AMS 700® or Titan Coloplast®) prosthesis and the size of the cylinders compared.

Results: During the reoperation, it was noticed a median upsize in the length of the cylinders of 1 cm in either one or both corporal bodies (range 0-2cm) as shown in Table 1. Fifty percent of the patients had deliberate undersizing at the initial operation due to a distal shunt. The mean IIEF-5 score preoperatively was 24 (range 23-25). After the initial insertion of the penile prosthesis the satisfaction rate according to the IIEF-5 was 80% compared to 90%, 3 months following the exchange of prosthesis.

Conclusion: Although some priapism patients have a shorter implant inserted initially due to previous shunt procedures, following a period of resolution the cylinders can be upsized at the elective exchange.

MP-06.16

Human Papilloma Virus Infection in Penile and Genitalia Tumors: Atypical Aspects

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Introduction and Objectives: One of the most interesting characteristics about the HPV infection is the polymorphism of the clinical appearance.

Materials and Methods: We prospectively started a study in 2002 to see the correlations between HPV infection and atypical clinical appearance at Urology and Dermatology Clinics. During 10 years we included in this analysis 128 patients with clinical condiloma acuminata of penile and genital areas.

Results: In 25 cases of the 128, we observed atypical clinical appearance photo documented: Atypical age: 4 cases (three children below 10 years old and one elder 84 years old); Atypical localization 4 cases: endo-urethral (one), external urethra meatus (two), sublingual (one); Atypical clinical subtypes found 17 cases: sub clinical types (4), Bowen papillomatosis (one) and together with invasive carcinoma (one), Eritroplasia Queyrat (3) and with simultaneous invasive carcinoma (one) pigmented condyloma acuminata (3), giant genital warts (2), Busche-Lowenstein condyloma acuminata (2). In one case we found a low risk infection that developed invasive carcinoma.

Conclusion: Male HPV infection takes a diversity of aspects regarding the localization, size, and evolution and transmission way of the lesion. This involves a careful attention in practice and a multidisciplinary approach of such cases. This requires a team management including urologists, dermatologists, pathologists and oncologists.

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The Effect of Acupuncture on Relieving Pain after Inguinal Surgeries

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Introduction and Objectives: Postoperative pain is one of the most prevalent and bothersome issues found in the surgical department. Nowadays, there are various methods of acupuncture used for relieving pain without the complications found in some routine postoperative analgesics. These methods could be especially useful for high risk patients prone to complications from analgesics, such as transplantation recipients. The aim of this study was to evaluate the efficacy of electro-acupuncture on postoperative pain control after inguinal surgeries.

Materials and Methods: Ninety male patients, who were referred to our department with indications of inguinal surgery, were included in the study and randomly divided into two groups, such as acupuncture and control. We used electroacupuncture for the acupuncture group and no actual acupuncture (but placed needle electrodes similar to the acupuncture group) for the control group. Postoperative pain was quantified by a blind observer in both groups using a visual analogue scale (VAS) standard score before being compared.

Results: Pain intensity and analgesic use were significantly higher in the control group ($P < 0.05$). In the acupuncture group, the VAS pain scores were significantly lower than the control group at 0.5, 1 and 2 hours post operation. When the opioid related side effects were compared for each group, the results showed that the number of subjects who experienced dizziness in the acupuncture group was significantly lower than the control group ($P < 0.05$).

Conclusions: Acupuncture in patients, after inguinal surgery, can reduce the need of analgesics, which also directly reduces the complications that may occur when analgesics are used in relieving pain postoperatively.

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Penis Rupture: A Topic for Interdisciplinary Consideration

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Introduction and Objectives: The purpose of the interdisciplinary cooperation between urological surgery and physics is the development of a physical simulation tool to be used by surgeons in order to give prognosis of possible penis rupture at a certain degree of deviation of the penis and to take prophylactic action.

Materials and Methods: For the physicist it

MP-06.15, Table 1.

Patients	Right corporal cylinder 1 st operation	Right corporal cylinder at exchange	Left corporal cylinder 1 st operation	Left corporal cylinder at exchange
1	19	19	18	19
2	21	22	20	22
3	21	23	21	23
4	22	23	22	23
5	21	22	21	22
6	18	21	19	21
7	22	23	22	23
8	23	23	22	23
9	20	22	20	22
10	21	22	21	22
Median	21	22	21	22